



## U-CLOSONE cream & ointment

**NAME AND STRENGTH OF ACTIVE SUBSTANCE:** Clobetasone Butyrate 0.05% w/w.

**PRODUCT DESCRIPTION:**

Cream: A white cream.

Ointment: White ointment.

Other ingredients for cream are Sodium Citrate, Chlorocresol, Glyceryl Monostearate, Glycerin, Cetostearyl Alcohol, Glyceryl Monostearate (and) PEG-100 Stearate, Dimethicone and Purified Water.

Other ingredients for ointment are Liquid Paraffin and White Soft Paraffin.

**PHARMACODYNAMICS:** Clobetasone Butyrate is a topically active fluorinated corticosteroid which has anti-inflammatory, anti-pruritic and vasoconstrictive actions.

**PHARMACOKINETICS:** Topical corticosteroids have anti-inflammatory, anti-pruritic and vasoconstrictive actions. When administered topically, particularly under occlusive dressings or when the skin is broken, sufficient corticosteroid may be absorbed to give systemic effects. Corticosteroids are extensively bound to plasmaproteins. Only unbound corticosteroids have pharmacological effects or are metabolised. They are metabolised mainly in the liver, also in the kidney, and are excreted in the urine.

**INDICATIONS:** All types of eczema and dermatitis, including atopic eczema, photodermatitis, otitis externa, primary irritant and allergic dermatitis, prurigo nodularis, seborrhoeic dermatitis and insect bite reactions. Maintenance between courses of more potent topical steroids in more resistant dermatoses.

**CONTRAINDICATIONS:** Hypersensitivity to Clobetasone Butyrate, to other corticosteroids or to any ingredient in this preparation, rosacea, acne vulgaris, peri-oral dermatitis, primary cutaneous viral infections (eg. herpes simplex, chicken pox). The use of U-Closone is not indicated in the treatment of primarily infected skin lesions caused by infection with fungi or yeasts (eg. candidiasis, tinea), or bacteria (eg. impetigo), peri-anal and genital pruritus.

**DRUG INTERACTIONS:** Unknown.

**ADVERSE EFFECTS:** Reported local adverse reactions are burning, itching, irritation, dryness, folliculitis, hypertrichosis, acneiform eruptions, hypopigmentation, maceration of the skin, secondary infection, skin atrophy, striae and milia. Large doses of corticosteroids may produce symptoms typical of hypoactivity of the adrenal cortex, with moon face, sometimes with hirsutism, buffalo hump, flushing, sometimes leading to a fully developed Cushing's syndrome.

**PRECAUTIONS/WARNINGS:** Appropriate treatment should be given in the presence of infections. If there is no favourable response, treatment with the corticosteroid should be discontinued until the infections has been controlled. Corticosteroid applied to the skin can be absorbed in sufficient amounts to produce systemic effects. Special care must be taken when treating paediatric patients as systemic absorption can occur in topical administration causing growth retardation. In infants and children, long-term continuous topical corticosteroid therapy should be avoided where possible, as adrenal suppression can occur even without occlusion. Thus suitable precautions should be taken during prolonged treatment, when treating extensive body surface areas, and when treating children (because of a larger skin surface area to body weight ratio). Suitable precautions should also be taken when treating stasis dermatitis and other skin diseases with impaired circulation. Discontinue use if irritation, sensitivity or other reactions develop and institute appropriate treatment. Use with care if applied on the eyelids as glaucoma might result. In infants the napkin may act as an occlusive dressing and increase absorption. Treatment should not normally exceed 7 days. Prolonged application to the face should be avoided. Long-term continuous topical therapy should be avoided. Do not use in or around the eyes.

**In Pregnancy:** Safety of its use during pregnancy has not been established. Topical steroids should not be used extensively in pregnancy, i.e. in large amounts or for prolonged periods.

**Nursing mothers:** It is not known whether topical administration of corticosteroids could result in sufficient systemic absorption to produce detectable quantities in breast milk. Nevertheless, caution should be exercised when topical corticosteroids are prescribed for a nursing woman.

**ROUTE OF ADMINISTRATION:** Topical.

**RECOMMENDED DOSAGE:** Apply up to four times daily until condition improves, then reduce frequency.

**OVERDOSE AND TREATMENT:** Corticosteroid applied to the skin can be absorbed in sufficient amount to produce systemic effect such as hypothalamic-pituitary-adrenal axis suppression, manifestation of Cushing's syndrome, hyperglycaemia and glucosuria. Tests which may be helpful in evaluating hypothalamic-pituitary-adrenal axis suppression include urinary free cortisol test and ACTH stimulation test. If hypothalamic-pituitary-adrenal axis suppression is found, then the drug should be withdrawn, frequency of application reduced or a weaker steroid used.

**PACKING:** 15g and 100g collapsible aluminium tubes.

**STORAGE:** Keep container well closed. Protect from strong light. Store below 30°C.

For external use only. Keep out of reach of children.

Recommended shelf-life: 3 years.

**MANUFACTURED BY :**

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